SRP documentation that survives review

D4341 and D4342 · what a reviewer needs before they can say yes

Scaling and root planing is denied more often than almost any other hygiene procedure, because the payer is looking for proof that this was treatment rather than cleaning. Build that proof at the visit, not after the denial.

AT THE VISIT

1. **Chart every tooth you treat.** Six points per tooth: probing depth, recession, clinical attachment loss, bleeding on probing, furcation where present, mobility. A note that says "deep pockets" is not a chart.
2. **Name the disease, not the code.** Stage and grade, which quadrants, which teeth. The code follows the chart — four or more teeth per quadrant is D4341, one to three is D4342. Count before you select.
3. **Say why it was therapeutic.** State that this treated active disease, and list the systemic conditions in the chart when they are documented. Preventive language invites a D1110 downgrade.
4. **Photograph and date what you saw.** Intraoral images of inflammation and bleeding, taken before the tissue calms down. Note the anaesthetic and the appointment length.

IN THE PACKET YOU SEND

- ☐ **Radiograph:** full-mouth series or bitewings showing bone loss that matches the charting, dated inside the payer window.
- ☐ **Chart page:** the six-point sheet for the treated teeth, not a summary screen.
- ☐ **Narrative:** teeth treated, findings, intent, and why the same date is not a prophylaxis.
- ☐ **Benefit check:** frequency left this benefit year, quadrant or calendar-year rule, and whether the benefits year is not the calendar year.

If it still denies: insufficient documentation means resend with the chart page first, not last. Frequency exhausted means re-check the benefit year before you appeal. Bundled with D1110 means prove the separate date and the therapeutic intent. Never resend the same packet unchanged.

BEANbite works denied SRP claims to resolution.

Send us the denial and the chart. We rebuild the packet, resubmit it, and work it through to payment.

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